

# The Global Burden of Mental Illness.

Prevalence, impact, and the treatment gap — global data (2023).

Audience: public health policy analysts & mental health program planners

Source: Our World in Data (2023)

Data: IHME, Global Burden of Disease (2025)

Measure used in this deck: DALYs per 100,000 people (world, 2023).

# Hundreds of Millions Affected

Lifetime risk of major depression is substantial.

**1 in 3**

**Women**

Will experience major depression in their lifetime.

**1 in 5**

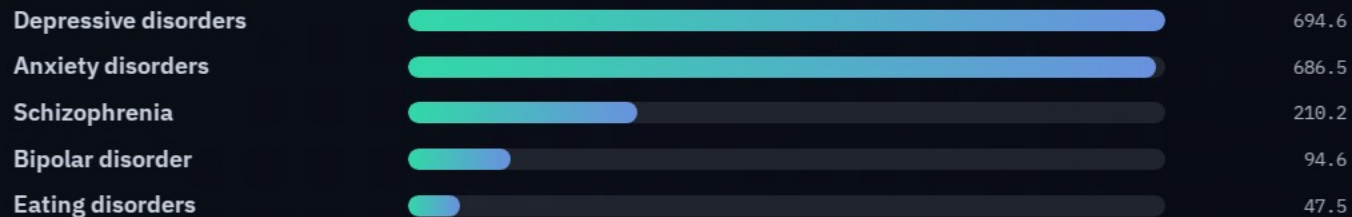
**Men**

Will experience major depression in their lifetime.

Mental health conditions span from common disorders (depression, anxiety) to less common but severe conditions (schizophrenia, bipolar disorder).

# Depression and Anxiety Dominate

World, 2023 • DALYs per 100,000 people (IHME GBD). Reference: figure\_1.



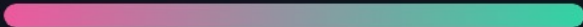
Depressive and anxiety disorders have nearly identical DALY rates (~695 and ~687).

# Depression and Anxiety Carry Nearly Equal Disease Burden

World, 2023 • DALYs per 100,000 people.

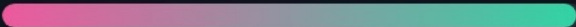
Depressive disorders

694.6



Anxiety disorders

686.5



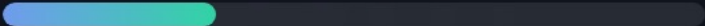
Difference: 8.1 DALYs per 100,000 (near-parity)

Together, depressive and anxiety disorders dominate the mental health disease burden across these categories.

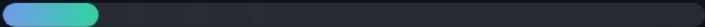
# Schizophrenia and Bipolar Disorder: Lower Prevalence but Severe Individual Impact

World, 2023 • DALYs per 100,000 people.

Schizophrenia 210.2



Bipolar disorder 94.6



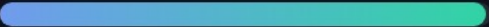
Population-level DALYs are lower than depression/anxiety, but disability can be severe for individuals.

## Context (dominant categories)

Depressive disorders 694.6



Anxiety disorders 686.5



Keep specialist pathways for severe conditions while scaling interventions for common disorders.

# Treatment Is Available but Often Lacking or Poor Quality

Mental illnesses are treatable — yet many people do not receive adequate care.



## Stigma & discomfort

Many people feel uncomfortable sharing symptoms with healthcare professionals or people they know.



## Access barriers

Treatment is often lacking, especially in low- and middle-income countries.



## Quality gaps

Even where treatment exists, it may be of poor quality.



## Underreporting

Discomfort with disclosure makes true prevalence difficult to estimate; burden may be higher than reported.

Implication for planners: improve help-seeking, access, and quality — while strengthening data collection.

# How People Cope: From Medication to Faith to Family Conversations

Global surveys show coping strategies vary widely across countries. Reference: figure\_2.



## Prescribed medication

Common in higher-income settings.



## Religious / spiritual activities

Prevalent in many regions worldwide.



## Talk to friends or family

A widely reported coping mechanism.

Policy implication: interventions should be culturally sensitive — avoid one-size-fits-all approaches.



# Key Takeaways: Prioritizing Depression, Anxiety, and Closing the Treatment Gap

- Depression and anxiety together represent the overwhelming majority of DALYs across these categories.
- Schizophrenia and bipolar disorder contribute fewer DALYs at the population level, but can impose severe disability on individuals.
- Effective treatments exist — yet access and quality remain inadequate in many settings.
- Stigma suppresses help-seeking and also makes true prevalence difficult to estimate (underreporting).
- Scale evidence-based interventions for common disorders while maintaining specialized care for severe conditions.

Source: Our World in Data • IHME GBD

World, 2023